

WILLOW CREEK MIDWIFERY

Childbirth Education Series

Handout: Understanding the Stages of Labor

Prepared by: Brian Santos, CNM

Last revised: January 2026

For use in Willow Creek childbirth education classes and prenatal visits.

This handout is for general educational purposes and does not replace individualized care.

OVERVIEW

Labor is the process by which your baby moves from inside your uterus into the world.

It happens in stages, and each stage has a different character. Understanding what to expect can help you feel less afraid and more prepared, though every labor is unique.

STAGE ONE: CERVICAL DILATION

This is the longest stage. The uterus contracts rhythmically, causing the cervix to soften, efface (thin out), and dilate (open) from 0 to 10 centimeters. Stage one has three phases.

Early labor (0 to 6 cm)

This is the warm-up. Contractions begin irregularly and gradually become more rhythmic.

Duration: Highly variable. First-time mothers may be in early labor for many hours.

What to do: Rest if it is nighttime. Eat light foods and drink water. Time contractions.

Call us when contractions are 5 minutes apart, lasting 1 minute, for at least 1 hour.

Active labor (6 to 10 cm)

Contractions become stronger, longer, and closer together. This is the hardest work.

Duration: Typically 3 to 8 hours for a first-time mother, often faster in subsequent births.

Coping: Change positions often. Try the birth tub. Use your breath. Lean on your support person.

Transition (8 to 10 cm)

The most intense phase, but also the shortest. Duration: Usually 30 to 60 minutes.

Many women feel nauseated, shaky, or overwhelmed at transition. This is normal.

STAGE TWO: PUSHING AND BIRTH

Your cervix is fully dilated. Now your baby moves through the birth canal.

You will feel a powerful, involuntary urge to push.

Duration: Anywhere from a few minutes to 2 to 3 hours.

Pushing positions: Upright, side-lying, hands and knees, supported squat, birth stool.

We encourage you to try different positions. There is no single right position.

Crowning: When your baby's head can be seen at the opening of the vagina.

Many women feel an intense burning or stretching sensation here. It is temporary.

Birth: We place your baby on your chest immediately if you and your baby are stable. Skin-to-skin time is protective and important. We will not rush it.

STAGE THREE: PLACENTA

After your baby is born, the uterus continues to contract and the placenta separates. Duration: Usually 5 to 30 minutes.

We offer delayed cord clamping (waiting until the cord stops pulsing before cutting). This is the default at Willow Creek unless there is a medical reason to clamp sooner. Your birth partner may cut the cord if they would like to.

STAGE FOUR: EARLY RECOVERY (FIRST TWO HOURS)

After the placenta is delivered, we stay with you for at least two hours. We monitor your bleeding, your blood pressure, your baby's temperature and nursing. We will help with the first latch if you are planning to breastfeed. This is a time for the three of you to be together. The room will be quiet.

QUESTIONS WE HEAR OFTEN

How will I know it is really labor?

Real labor contractions become progressively stronger, longer, and more frequent over time. If you are unsure, call us. That is what we are here for.

What if I want pain medication?

We support every choice you make about your care. If you decide you want an epidural or other pain medication, we will help you transfer to a hospital where that is available. There is no judgment. Asking for what you need is not giving up.

What if something goes wrong?

We are trained to recognize and respond to complications. We have a direct relationship with Ridgeview Medical Center and Dr. Ransom for any birth that needs hospital support. Transfers are not failures. Transfers are good medicine.

For questions about anything in this handout, bring them to your next prenatal visit.

Willow Creek Midwifery
240 Merrimon Avenue, Asheville, NC 28801
828-555-0155
willowcreekmidwifery.com